

The Reset: NSW Health's Workforce After the Wage Decision

Cycle: 18 May 2026 · Audience: NSW Health Future Health Workforce Planning leadership · Horizon: 2 to 5 years

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Executive Synthesis

The cycle's central tension is that three forces NSW Health was treating as separable have become a single workforce problem inside thirty days. The Industrial Relations Commission's 16 April 2026 decision ([NSW IRC](#), April 2026) delivered a staged 16% pay reset for registered nurses and midwives, 18% for enrolled nurses and 28% for assistants in nursing, backdated to 1 July 2025. The Single Digital Patient Record went live at Justice Health and NSW Health Pathology on 25 March 2026 ([NSW Government](#), March 2026) and is moving through Hunter New England (25,000 staff in scope) during May 2026. And the Therapeutic Goods Administration's mandatory adverse-event reporting regime for AI-enabled medical devices commenced on 21 March 2026 ([TGA](#), March 2026). Each on its own is manageable. Arriving together, they reshape the FY27 budget, the digital deployment roster and the clinical governance burden in the same quarter.

Three structural shifts define the workforce environment NSW Health is entering. First, the wage-cost pressure that pipeline-and-retention pressure has been building toward through 2025-26 has converted from contingent to binding: the IRC decision delivers a structural pay-base shock with cross-cadre relative compression, meaning the pay gap between nursing grades has narrowed (assistants in nursing receive a 28% uplift over three years against 16% for registered nurses), and the union has signalled the year-one 10% reset will not meet cost-of-living or retention pressures ([NSWNMA](#), April 2026). Second, AI-augmented clinical work has moved from "adoption" to "pilot-to-mandate": the TGA reporting regime, AHPRA's standing accountability guidance ([AHPRA](#), evergreen) and an OECD scale-up framework published in March ([OECD](#), March 2026) collectively shift the live question from whether to deploy ambient AI scribes to who carries the medico-legal weight when they do. Third, scope-of-practice implementation

has begun in earnest: the 1 March 2026 MBS updates restructured the Aboriginal and Torres Strait Islander health-assessment items ([NACCHO](#), March 2026), giving the Cormack agenda a concrete operating handhold inside the ACCHO sector.

The spine of this cycle is Cluster 1, the wage reset. It is not the largest workforce story by long-run impact (Cluster 4, attrition, retains that claim) but it is the cycle's organising shock: every other cluster's economics now run through the changed pay base. The Productivity Commission's Report on Government Services 2026 already shows medical-practitioner attrition climbing to 24.2% ([Productivity Commission](#), February 2026); the wage reset addresses one cadre's pressure but leaves the medical, allied health and ACCHO workforces inside the same operating envelope without the same uplift. Skill-mix economics, agency reliance, and the internal locum agency stood up at \$9.5 million in the 2025-26 Budget ([NSW Treasury](#), June 2025) all need to be reweighted against new relativities by the FY27 budget round.

Every other cluster's economics now run through the changed pay base.

Five forces are dominating leadership attention this cycle. The pay-base shock and its relativity compression. The SDPR rollout, where leaked Hunter New England readiness concerns ([Health Services Daily](#), March 2026) and frontline reports of poor communication and slipping go-live timing ([Health Services Daily](#), May 2026) sit alongside one of the largest coordinated training programs NSW Health has attempted (around 21,000 staff trained, 1,900 classes) ([eHealth NSW](#), February 2026). The AI pilot-to-mandate inflection. Early-career and rural attrition. And the scope-of-practice unfreeze, where AMA's GP-led framing ([AMA](#), April 2026) and ACN's independent-NP framing ([ACN](#), January 2026) now address the same MBS reform terrain from different positions.

The insight likely to surprise leadership is the inter-cadre relativity arithmetic. The 28% AIN uplift against 16% for RNs is not an internal HR detail: it changes the marginal cost of every skill-mix decision LHDs make, it creates the conditions for a credible RN relativity case the year-one 10% reset will not by itself settle, and it creates a work-value precedent ([6 St James Hall](#), April 2026) that other female-dominated NSW public-sector classifications will read closely. The risk is not the wage rise; it is the second-order industrial dynamic the relativity compression will set running through 2026-28.

Where this analysis could be wrong

The reading rests on one load-bearing claim: that the IRC decision is a one-off work-value reset rather than the first of a cascade of comparable decisions across female-dominated NSW public-sector classifications. If a second NSW IRC decision lands within twelve months extending the work-value reasoning to, for example, allied-health classifications or public-sector teachers, the fiscal envelope for Cluster 1 widens materially and the FY27 budget arithmetic the Strategic Implications assume falls apart. The evidence that would force a material revision is a second NSW IRC work-value award citing Justice Taylor's reasoning at scale before mid-2027, or a NSW Government decision to extend the staged 16/18/28% reset architecture beyond nursing and midwifery. Until then, the report treats the decision as a single, large, contained shock.

The decisions that cannot be deferred this cycle

1. **Commit a re-based FY27 workforce financial plan that absorbs the IRC reset and re-prices skill-mix arithmetic.** Without an explicit re-pricing, LHDs will default to status-quo skill-mix and silently absorb a structural margin shock through agency and overtime.

Lead-check. *Proposed lead: Future Health Workforce Planning with NSW Treasury and Ministry CFO as joint owners. Resourcing: significant, full FY27 budget cycle workstream. Dependencies: confirmed backdating cost from Treasury; LHD-level skill-mix baseline data from eHealth NSW HR systems.*

2. **Authorise a single SDPR workforce-stabilisation playbook before the next Tranche cut-over.** Hunter New England has surfaced communication, training and infrastructure gaps that will repeat at every subsequent LHD without a codified playbook owned at Ministry level.

Lead-check. *Proposed lead: SDPR Implementation Authority co-led with HETI and the Ministry Workforce Branch. Resourcing: stretch, requires reallocation from existing program-management capacity. Dependencies: SDPRIA willingness to publish a candid post-go-live review; HETI capacity to recycle training content across LHDs.*

3. **Decide the NSW Health position on ambient AI scribe adoption before TGA enforcement activity begins.** The TGA mandatory-reporting regime commenced 21 March 2026 and a NSW Health position is needed to direct LHD procurement, indemnity and governance choices coherently.

Lead-check. *Proposed lead: Future Health Workforce Planning with eHealth NSW, ACI and the Office of the Chief Medical Officer. Resourcing: incremental on policy, stretch on the workforce-development response. Dependencies: AHPRA clarification on profession-specific application of standing guidance; indemnifier alignment with NSW Health risk appetite.*

4. **Confirm the scope-of-practice reform posture NSW Health will defend in the Commonwealth FY27 cycle.** The Cormack implementation phase is open and ACN, ANMF, AMA and RACGP positions now diverge; without a stated NSW posture, the State will be a price-taker in a debate that reshapes its own multidisciplinary economics.

Lead-check. *Proposed lead: Future Health Workforce Planning with ACI and Ministry Policy Branch. Resourcing: incremental. Dependencies: Commonwealth response to Cormack implementation timetable; NSW Health stakeholder consultation including AH&MRC and Pharmacy Guild.*

Each is developed below, with a decision posture, in the four Strategic Implications.

Workforce Snapshots

Four lenses on the same cycle. Each card surfaces the one question this cycle puts to that workforce category, anchored on the evidence base.

NURSING & MIDWIFERY

Will the year-one 10% reset hold the line on attrition, or is the union right that it will not?

The shift: The IRC decision delivers the largest single nursing-pay uplift in NSW history, with Justice Taylor citing the historical undervaluation of caring and interpersonal skills given the ~90% female composition of the workforce ([ANMJ](#), April 2026). NSWNMA welcomes the recognition but warns that 10% in year one falls short of cost-of-living and retention need.

The question to brief: What attrition assumptions does the FY27 workforce plan adopt, given Productivity Commission RoGS 2026 shows nursing and midwifery attrition at 24.5% even before the reset takes full effect ([Productivity Commission](#), February 2026)?

MEDICAL

How does NSW Health hold medical retention when the cadre next to it has just been re-priced?

The shift: Medical-practitioner attrition rose to 24.2% nationally in RoGS 2026, from 22.9% in 2023 ([Productivity Commission](#), February 2026). At the same time, GP AI-scribe use has doubled inside a year to around 40% of practices ([RACGP newsGP](#), March 2026), reshaping clinical documentation expectations across the medical workforce.

The question to brief: Does the FY27 plan rely on the Rural Generalist Single Employer Pathway intake of 24 doctors ([NSW Government](#), February 2026) and other supply levers, or does it accept that an explicit medical retention posture is now overdue?

RURAL AND REMOTE

Does NSW back the CRANaplus generalist pathway, or rely on the existing piecewise architecture?

The shift: Region of employment is now confirmed as a material driver of intention-to-remain ([Journal of Nursing Management](#), February 2026), with mentorship and reflective practice the strongest retention effects. CRANaplus has proposed a national generalist rural-and-remote nursing and midwifery pathway delivering 1,000 accredited generalists by 2031 ([CRANaplus](#), January 2025).

The question to brief: Does NSW Health back the CRANaplus pathway through HETI as an FY27 commitment, layered on the Rural Generalist Single Employer Pathway, or hold the current piecewise architecture?

ABORIGINAL COMMUNITY CONTROLLED SECTOR

Does NSW Health treat the 1 March MBS reset as a workforce-multiplier or an MBS housekeeping change?

The shift: The 1 March 2026 MBS updates removed age-based clinical activities from the Aboriginal and Torres Strait Islander health-assessment items 715, 218, 92004, 92011, reframing the assessment around clinical need across the life course ([MBS Online](#), March 2026). Aboriginal Community Controlled Health Organisations are positioned as the best-practice multidisciplinary model ([Croakey](#), March 2026).

The question to brief: Does NSW Health treat the MBS change as a quiet workforce-multiplier (and resource ACCHOs to act on it), or as a Commonwealth MBS housekeeping change with no NSW workforce implication?

Signal Clusters

The cycle's signals are organised into five clusters, ranked by impact on NSW Health (Future Health Workforce Planning)'s near-term decisions. **Immediate:** changes the FY27 budget cycle, the corporate structure or the proposition. **Near-Term:** changes NSW Health (Future Health Workforce Planning)'s competitive position over the next twelve months. **Longer-Range:** a multi-year structural factor to track and revisit each cycle.

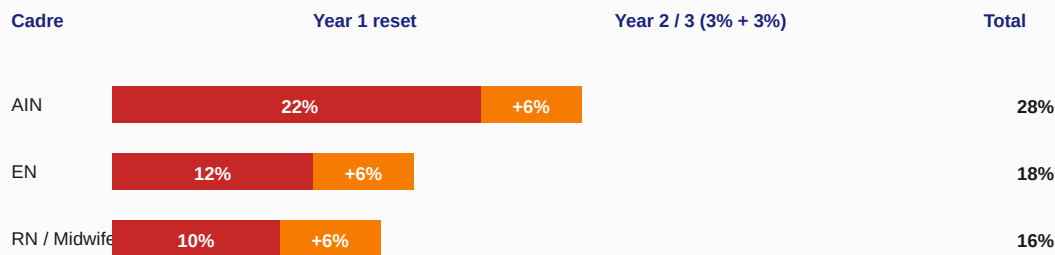
1. The Wage Reset

IMMEDIATE

The IRC's 16 April 2026 decision is a structural pay-base shock at a scale NSW Health has not absorbed in many years. It re-prices every skill-mix decision LHDs make, it re-anchors the FY27 budget, and it sets a work-value precedent that other female-dominated NSW public-

sector classifications will read closely. The leadership confrontation is not the increase itself; it is that the cross-cadre relativity arithmetic gives NSWNMA a credible second front, and that the year-one 10% reset will arrive into the same cost-of-living and burnout environment that drove RoGS 2026's attrition reading.

Figure 1. Relativity compression: staged uplifts by cadre, 1 July 2025 to 30 June 2028



Staged award per NSW IRC, NSWIRComm 4 (16 April 2026); backdated to 1 July 2025.

Year-one reset plus 3% in each of years two and three, per NSW IRC NSWIRComm 4 (16 April 2026).

- The IRC's staged award delivers 16% to registered nurses and midwives, 18% to enrolled nurses and 28% to assistants in nursing across three years from 1 July 2025, with year-one resets of 10/12/22% and 3% in each of the following two years ([NSW IRC](#), April 2026).
- Justice Ingmar Taylor's reasoning explicitly cites the historical undervaluation of caring and interpersonal skills in a workforce around 90% female, opening a work-value precedent that legal commentators flag as portable to other female-dominated NSW public-sector classifications ([6 St James Hall](#), April 2026).
- The Minns Government has accepted the decision and committed to back-pay from 1 July 2025, characterising it as the largest single uplift in NSW nursing pay in history ([NSW Government](#), April 2026), while the Opposition has signalled it will monitor any compensating cuts to regional services or capital programs ([NSW Liberal Party](#), April 2026).
- The 2025-26 NSW Budget had already allocated \$9.5 million to stand up an internal NSW Health locum agency to reduce agency reliance, inside a \$35.1 billion health envelope ([NSW Treasury](#), June 2025), an architecture whose marginal economics now shift under the new pay base.

Inter-cadre relativity compression: the second-order industrial dynamic

SPOTLIGHT

Read at face value, the IRC award is a single decision. Read at the level of skill-mix economics, it is three different decisions that diverge sharply: assistants in nursing receive a 28% uplift while registered nurses and midwives receive 16%, compressing the relative price of substitution along the nursing skill ladder by twelve percentage points over three years ([Hospital + Healthcare](#), April 2026). For LHDs the marginal-cost arithmetic changes in two directions at once: the AIN role becomes more attractive to deploy at the bedside, while the RN differential narrows enough to give NSWNMA grounds for a relativity claim that the year-one 10% reset will not by itself settle. The union has signalled exactly this, calling the deal historic in scale but warning it leaves NSW RNs behind other states ([NSWNMA](#), April 2026). The strategic question is not whether the relativity dynamic will surface, but when, and which forum: another NSWIRC work-value case, an enterprise-bargaining round, or a Ministry-led skill-mix re-design.

Counter-argument

The strongest challenge to a "wage-shock cascade" reading is that the IRC's reasoning was tightly grounded in the specific evidentiary record on nursing and midwifery work value, not a portable principle of broader gendered-undervaluation. On that reading, the decision is contained, comparable awards across other classifications are years away if they come at all, and the relativity compression is a one-cycle adjustment NSW Health can absorb through normal enterprise bargaining. The harder counter is that the Minns Government's public framing as "historic" makes any future restraint posture costly to communicate, which both extends the political life of the award and makes a second decision in the next twelve months more salient than the legal reasoning alone would suggest.

Decision link: Strategic Implications [1](#) (FY27 re-base) and [4](#) (work-value precedent watch).

2. SDPR go-live and the digital-deployment workforce shock

IMMEDIATE

SDPR has moved from program to operating reality. Tranche A is live at Justice Health and NSW Health Pathology, with Hunter New England LHD (around 25,000 staff in scope) cutting over in May 2026 on a schedule the Implementation Authority itself describes as having "critical foundations" still being laid ([Health Services Daily](#), March 2026). The leadership confrontation is that NSW Health is running one of the largest coordinated workforce-affecting

digital deployments it has attempted at the same time as the wage reset is being absorbed, with public signals that Hunter New England is not ready, and that this pattern will recur at every subsequent LHD without a codified workforce-stabilisation playbook.

- The Single Digital Patient Record went live at Justice Health and NSW Health Pathology (John Hunter Hospital Lab) on 25 March 2026, the first of a sequential rollout intended to reach all 17 LHDs and specialty networks by end-2028 ([NSW Health](#), evergreen).
- Hunter New England and the remaining NSW Health Pathology HNE sites are scheduled for May 2026 cut-over, with more than 25,000 staff in scope, in what specialist coverage characterises as one of the largest coordinated digital deployments NSW Health has attempted ([Pulse+IT](#), March 2026).
- Internal SDPR briefing notes dated 11 March 2026 acknowledge the system build is not fully complete and that significant numbers of staff still require training; the HNE LHD May 2026 go-live is flagged as at risk because of infrastructure variability, training shortfalls and cybersecurity concerns ([Health Services Daily](#), March 2026).
- Frontline HNE clinicians have publicly described scant communication and missing guidance ahead of go-live, with Dr Teresa Anderson reframing the schedule to "toward the middle of the year" ([Health Services Daily](#), May 2026), even as eHealth NSW reports that around 21,000 staff have been trained across 1,900 classes, with 200 LHD volunteers supporting delivery ([eHealth NSW](#), February 2026).

Counter-argument

The frontline accounts are genuine but partial: they come from clinicians under acute pre-go-live load, the most disconfirming sources are paywalled trade press relying on anonymous voices, and the structured training data (around 21,000 trained, 1,900 classes) shows one of the largest coordinated learning programs NSW Health has attempted is in fact in motion. The realistic counter is that SDPR will land messily but successfully at Hunter New England with a short, recoverable productivity dip, in which case the workforce-stabilisation playbook proposed here is over-built for the actual risk and constitutes an avoidable governance overhead.

Decision link: Strategic Implications [2](#) (SDPR workforce-stabilisation playbook) and [1](#) (FY27 cost-base).

3. AI-augmented clinical work moves from pilot to mandate

NEAR-TERM

The AI question inside NSW Health has shifted from adoption to accountability. The TGA's mandatory adverse-event reporting regime for medical devices (including AI-enabled software-as-a-medical-device) commenced 21 March 2026; AHPRA's standing guidance makes practitioners personally accountable for AI-augmented decisions; ambient AI scribes

are now used by around 40% of Australian general practices. The leadership confrontation is that NSW Health needs an explicit ambient-AI-scribe deployment posture before LHDs and individual clinicians establish a de facto standard through procurement and the indemnity market.

- From 21 March 2026 healthcare-facility reporting of medical-device adverse events, including for AI-enabled software-as-a-medical-device, becomes mandatory in Australia, with deaths or serious deteriorations to be reported within 10 days ([TGA](#), March 2026); legal analysis warns sponsors and healthcare organisations must maintain ongoing post-market surveillance as AI products iterate through retraining ([Allens](#), April 2026).
- AHPRA's standing guidance holds practitioners professionally accountable for AI-augmented decisions, requiring human judgment, understanding of training and intended use, informed consent for material AI use, and privacy and indemnity compliance ([AHPRA](#), evergreen).
- RACGP newsGP polling shows GP AI-scribe use rising from 22% (August 2024) to 40% (November 2025); around 70% of patients are comfortable with use in consult; only 25% of practices have an AI governance policy and only 42% of GPs know their indemnifier's AI position ([RACGP newsGP](#), March 2026); independent reporting corroborates the adoption figure and confirms that the binding constraints are now governance, workflow integration and PMS integration rather than tool availability ([Medical Republic](#), March 2026).
- The OECD's March 2026 scale-up report finds AI is used in health systems across all OECD countries but national-level scale-up remains limited, with only around 10% of medical-imaging applications used at national scale; it identifies fragmented data, regulatory uncertainty and workforce-capacity gaps as the binding constraints ([OECD](#), March 2026), echoing the earlier OECD finding that the fastest-growing digital-skill demands in health occupations are health information management, telehealth and cybersecurity ([OECD](#), May 2025).

The ambient AI scribe legal-liability frontier

SPOTLIGHT

Australian medico-legal commentary is now explicit that GPs adopting AI scribes must consult indemnity insurers, ensure Australian-law-compliant data handling, and review outputs for false positives and negatives before sign-off, with emerging case law and indemnifier guidance shaping standard-of-care expectations ([RACGP / AJGP](#), May 2025). Focus-group research with 33 Australian medical practitioners identifies insufficient AI knowledge, medico-legal risk and privacy concerns as the primary barriers to ambient scribe adoption ([Healthcare IT News](#), April 2026). For NSW Health, the unresolved question is whether ambient-scribe use in public hospital settings travels under the same indemnity architecture as general practice. If LHDs procure ambient-scribe tools at scale without an explicit NSW Health position on data residency, consent script and clinician sign-off discipline, early adverse-event reports under the TGA regime are likely to begin shaping a de facto NSW standard that the Ministry will then have to react to rather than help define.

Counter-argument

The TGA, AHPRA and OECD pieces taken together can be read as governance scaffolding around a still-small-scale clinical change, not a "mandate" inflection. Ambient-scribe penetration in NSW public hospitals is meaningfully lower than in general practice; the TGA reporting regime applies to a wider universe of medical devices than AI alone; and AHPRA's guidance is in substance an extension of long-standing professional-accountability principles. On this reading, a forward-leaning NSW Health AI-scribe posture is premature, and the right move is to monitor the first six months of TGA-regime reporting and let national governance settle before committing NSW Health to a position.

Decision link: Strategic Implications [3](#) (NSW Health ambient-AI-scribe posture) and [4](#) (monitor).

4. Pipeline and retention: attrition climbs, early-career exit accelerates

NEAR-TERM

The structural workforce shortfall is moving in opposite directions across cadres. Medical-practitioner attrition is rising, nursing attrition is easing from a high base, and intent-to-leave signals concentrate in the early-career and rural cohorts where mentorship, reflective practice and region-of-employment are now confirmed as the binding levers. The leadership confrontation is that the FY27 plan needs to back specific pipeline architectures (Rural Generalist intake, CRANApplus pathway, HETI scholarships) rather than treat pipeline as a residual.

- Report on Government Services 2026 (released February 2026) records medical-practitioner attrition at 24.2%, up from 22.9% in 2023; nurses and midwives at 24.5%, down from 28.9%; and a national workforce of 122,749 FTE medical practitioners (including 815 Aboriginal and Torres Strait Islander) and 372,821 FTE nurses and midwives in the eighth consecutive year of growth ([Productivity Commission](#), February 2026).
- AHPRA's Workforce Retention and Attrition Project finds that across nine regulated professions registered practitioners per 100,000 rose 29.6% over 2014-2023 but replacement rates fluctuate, with 79% of practitioners intending to maintain registration and more than 12% unsure or planning to leave, for reasons led by mental burnout, retirement, feeling undervalued and lack of professional satisfaction ([AHPRA](#), February 2025); peer-reviewed analysis confirms the longitudinal pattern ([Australian Health Review](#), June 2025).
- Region of employment is now a confirmed material driver of nurse intention-to-remain in Australia, with mentorship, reflective practice, resilience training and career coaching showing the strongest retention effects, particularly in rural and remote settings ([Journal of Nursing Management](#), February 2026).

- NSW Government announced 24 additional doctors entering the NSW Rural Generalist Single Employer Pathway 2026 intake, employed on the NSW Health Medical Officer Award and able to work across eight rural LHDs ([NSW Government](#), February 2026); HETI's 2025-26 Performance Agreement commits \$8.4 million in scholarships to 1,842 students and \$0.7 million in rural and regional scholarships to 232 students ([HETI](#), July 2025).

The early-career exit cohort the headline numbers obscure

SPOTLIGHT

The aggregate RoGS 2026 figure of 24.5% nursing and midwifery attrition is a smoothed total that masks an early-career exit pattern surfacing in the peer-reviewed and peak-body evidence. ANMF advocacy now frames the binding pipeline constraint as the financial and time burden on student nurses during clinical placements, and is pushing for federally mandated minimum staffing embedded into the National Safety and Quality Health Service standards ([ANMF](#), January 2026); ANMF's 2026 priorities extend this to scope-of-practice and the Maternity Futures Report ([ANMJ](#), February 2026). The implication for NSW Health is that the IRC pay reset will not in itself fix the early-career exit cohort; the structured interventions that the evidence base supports (mentorship, reflective practice, placement support, region-specific career coaching) sit outside the wage decision and need to be authorised separately, by HETI, inside the FY27 cycle.

Counter-argument

The IRC pay reset is itself a large-scale retention intervention at a scale NSW Health has not made in many years; once it flows through the pay packet in late 2026, it may move the early-career exit cohort more than the surveyed exit-driver list suggests, because cost-of-living pressure is the binding constraint that AHPRA's "feeling undervalued" category understates. On this reading, layering a HETI-led pipeline architecture on top of the wage reset is sequencing error: NSW Health should give the pay reset a full year to do its work before committing fresh capital to pipeline reform, and use FY27 to baseline rather than to act.

Decision link: Strategic Implications [1](#) (FY27 cost-base) and [4](#) (early-career attrition watch).

5. Scope-of-practice unfreezes; multidisciplinary care economics

NEAR-TERM

Scope-of-practice reform has moved from review to implementation. The 1 March 2026 MBS updates restructured Aboriginal and Torres Strait Islander health-assessment items; ACN is pressing the Commonwealth on nurse-practitioner direct referral and RN prescribing; AMA is

reframing the debate around GP-led practice-nurse roles within clinical governance, and RACGP is pressing for a measured, evidence-based approach to pharmacist-scope expansion. The leadership confrontation is that NSW Health needs an explicit posture into the Commonwealth FY27 cycle, because the Cormack Review's productivity dividend is now actively being claimed by treasurers as a national-productivity reform ([Croakey](#), February 2026).

- The Cormack Scope of Practice Review's 18 recommendations across workforce design, development, education and planning remain the canonical reform reference, finding that virtually all primary-care health professionals face restrictions to working at full scope unrelated to their education or competence ([Department of Health](#), October 2024).
- The 1 March 2026 MBS updates removed age-based clinical activities from Aboriginal and Torres Strait Islander health-assessment items 715, 218, 92004 and 92011, reframing the assessment around clinical need across the life course ([NACCHO](#), March 2026; [MBS Online](#), March 2026).
- ACN's 2026-27 Pre-Budget Submission prioritises Cormack implementation steps including enabling nurse practitioners and endorsed midwives to refer directly to specialists, legislating qualified RN prescribing, authorising NPs and qualified RNs to order diagnostic tests, and removing the collaborative-arrangement requirement ([ACN](#), January 2026); AMA's April 2026 position carefully reframes the debate around GP-led practice-nurse roles inside clinical governance, with practice-level MBS items for nurse-delivered care under GP oversight ([AMA](#), April 2026); RACGP's March 2026 release on NT pharmacist scope urges a measured, evidence-based approach with GP collaboration ([RACGP](#), March 2026).
- NSW Agency for Clinical Innovation's Rehabilitation Model of Care positions multidisciplinary teams as central to the future state-wide model, removing silos and tailoring to local priorities ([NSW ACI](#), March 2026); cross-sector commentary highlights that around 30% of nurses report not working to full scope and that ACCHOs are best-practice multidisciplinary models for the reform ([Croakey](#), March 2026).

The NACCHO MBS reset as a quiet workforce-multiplier

SPOTLIGHT

The 1 March 2026 MBS change reads as administrative housekeeping. Treated as a workforce signal, it is something larger. Removing the age-bracket structure from items 715, 218, 92004 and 92011 lets ACCHOs and GPs design Aboriginal and Torres Strait Islander health assessments around clinical need rather than age category, which in turn extends what an Aboriginal Health Practitioner, an Aboriginal Health Worker, a practice nurse or a nurse practitioner can be deployed to do under the same item suite. That is a workforce-multiplier inside the ACCHO sector specifically, the sector NSW Health relies on for its Closing the Gap delivery credibility, and the same sector Croakey identifies as the best-practice multidisciplinary model for the broader Cormack reform. The strategic question for NSW Health is whether to actively resource the ACCHO sector to act on the new item architecture, or to treat the MBS change as a Commonwealth matter without NSW workforce implication.

Counter-argument

The Cormack agenda is canonical but slow-moving: the most consequential implementation steps require Commonwealth legislation, state regulator alignment and indemnity-market acceptance, none of which will resolve before FY28. A NSW Health "scope-of-practice posture" risks committing the State to specific reform positions on a Commonwealth timetable NSW does not control, and on positions (independent NP referral, RN prescribing) that NSW's own clinical-governance bodies and the medical peak have not yet aligned on. The more disciplined move is to engage actively in the Commonwealth process but defer a public NSW posture until the legislative pathway clarifies.

Decision link: Strategic Implications 3 (scope posture into Commonwealth cycle) and 4 (monitor relativity precedent).

Strategic Implications

Four decisions turn this cycle's signals into the FY27 plan. Each names the move, a horizon and a decision posture. Opportunities (the ACCHO MBS workforce-multiplier; the NSW AI-scribe posture as a national lead) are folded into Prepare-posture implications, not split off as a separate section.

SI 1: Re-base the FY27 workforce financial plan around the IRC reset

The Future Health Workforce Planning leadership, jointly with NSW Treasury and the Ministry CFO, should deliver a re-based FY27 workforce financial plan by the FY27 budget submission window in late 2026 that absorbs the staged 16/18/28% IRC award, re-prices the skill-mix arithmetic LHDs use against the new relativities, and re-tests the \$9.5 million internal locum-agency architecture under the new pay base. Without this re-base, LHDs will default to status-quo skill-mix and the structural margin shock will silently surface as agency and overtime expenditure through FY27 and into FY28.

Action: a re-based FY27 workforce financial plan endorsed jointly by the Ministry CFO and the Future Health Workforce Planning leadership, into the FY27 budget submission cycle Q3 2026.

DECIDE

Lead-check. *Proposed lead: Future Health Workforce Planning, with Ministry CFO and NSW Treasury as joint owners. Resourcing: significant, full FY27 budget cycle workstream. Dependencies: confirmed backdating cost from Treasury; LHD skill-mix baseline data from eHealth NSW HR systems.*

Draws on Clusters 1 and 4.

SI 2: Authorise a single SDPR workforce-stabilisation playbook owned at Ministry level

The SDPR Implementation Authority, co-led with HETI and the Ministry Workforce Branch, should publish a codified workforce-stabilisation playbook before the next Tranche cut-over, covering communication cadence, super-user redeployment from completed LHDs, extended at-the-elbow support beyond the standard three weeks, and a Ministry-owned post-go-live workforce review at each LHD. The Hunter New England experience should be captured candidly while memory is fresh; the cost of not codifying is that each subsequent LHD wave repeats the same coordination failures with diminishing political tolerance.

Action: a published SDPR workforce-stabilisation playbook, owned by the Ministry Workforce Branch and signed off by SDPRIA, before the next LHD Tranche go-live.

PREPARE

Lead-check. *Proposed lead: SDPR Implementation Authority co-led with HETI and the Ministry Workforce Branch. Resourcing: stretch, requires reallocation from existing program-management capacity. Dependencies: SDPRIA willingness to publish a candid HNE post-go-live review; HETI capacity to recycle training content across LHDs.*

Draws on Cluster 2.

SI 3: Set a NSW Health ambient-AI-scribe deployment posture and a scope-of-practice posture into the Commonwealth FY27 cycle

The Future Health Workforce Planning leadership, with eHealth NSW, ACI and the Office of the Chief Medical Officer, should agree a NSW Health ambient-AI-scribe deployment posture by end-Q3 2026 covering data residency, consent script, clinician sign-off discipline and the indemnity architecture for public-hospital use, before TGA-regime reporting and the indemnifier market set a de facto NSW standard by accident. In parallel, NSW Health should agree the scope-of-practice posture it will defend into the Commonwealth FY27 cycle, including how it will resource ACCHOs to act on the 1 March 2026 MBS changes, before AMA, ACN and RACGP positions consolidate the national debate without NSW input.

Action: a documented NSW Health ambient-AI-scribe deployment posture endorsed by the Office of the Chief Medical Officer by end-Q3 2026, plus a scope-of-practice position paper submitted to the Commonwealth FY27 cycle by the same window.

PREPARE

Lead-check. *Proposed lead: Future Health Workforce Planning, with eHealth NSW, ACI and the Office of the Chief Medical Officer. Resourcing: incremental on policy, stretch on the workforce-development response. Dependencies: AHPRA clarification on profession-specific application of standing guidance; indemnifier alignment with NSW Health risk appetite; AH&MRC and Pharmacy Guild consultation.*

Draws on Clusters 3 and 5.

SI 4: Establish a quarterly work-value precedent and early-career attrition watch

The Ministry's industrial-relations function, with Future Health Workforce Planning, should stand up a quarterly watch on two named signals through to FY28: any second NSW IRC work-value award citing Justice Taylor's reasoning at scale; and the early-career nursing exit cohort within the first 1-4 years of practice as reported in the next AIHW and AHPRA workforce updates. Both signals are leading indicators of whether the FY27 re-base and the structural retention assumptions need a within-cycle revision. Watch is the right posture: the signals are not yet sufficient to act on, but the cost of missing them once they materialise is the cost of re-baselining FY28 mid-cycle.

Action: a quarterly watch brief on the two named signals from the Ministry's IR function, first edition end-Q3 2026, persistent through FY28.

MONITOR

Lead-check. *Proposed lead: Ministry industrial-relations function with Future Health Workforce Planning. Resourcing: incremental. Dependencies: AIHW Q3 2026 workforce update tracking early-career exit cohort; access to NSWIRC case-list monitoring.*

Draws on Clusters 1, 4 and 5.

Scenario Matrix

The 2x2 takes the two uncertainties that will shape NSW Health's 2027-28 workforce decision window more than any other: whether the IRC reset is contained or cascades through other female-dominated NSW public-sector classifications; and whether the SDPR rollout stabilises into a repeatable pattern or runs into compounding workforce-readiness slippage across LHDs. The four scenarios below are planning aids, not forecasts.

Contained Reset, Stable Rollout

The IRC decision remains a single contained award, no comparable work-value cascade emerges by mid-2027, and SDPR Tranche A and B land messily but recoverably. The FY27 re-base absorbs cleanly, the relativity-compression dynamic is managed through normal enterprise bargaining, and the SDPR workforce-stabilisation playbook becomes a Ministry standard that subsequent LHD waves run on. NSW Health gets a one-cycle window to reinvest in pipeline architecture (CRANaplus, HETI scholarships, Rural Generalist pathway) and to set a forward-leaning ambient-AI-scribe posture. The strategic risk in this scenario is complacency: the pay reset masks the early-career attrition signal long enough for it to harden.

Early indicators: no second NSW IRC work-value award by Q2 2027; SDPRIA publishes a candid HNE post-go-live review; AIHW Q3 2026 update shows early-career exit cohort stabilising.

Cascade Reset, Stable Rollout

A second NSW IRC work-value award lands inside twelve months, extending the reasoning to one or more other female-dominated public-sector classifications. SDPR rolls out broadly to schedule. The fiscal envelope widens materially, the FY27 re-base assumed in SI 1 is overtaken inside the cycle, and the Ministry faces a wage-and-precedent debate that crosses health, education and human services. NSW Health's relative position is preserved on the technology and operating-model fronts, but the cost-base shock dominates leadership attention through FY28. The strategic posture pivots to fiscal containment and explicit cross-portfolio coordination with Treasury.

Early indicators: a second NSWIRC work-value matter listed for hearing by end-2026; Treasury revising its wage parameter mid-cycle; NSW Government extending the staged-reset architecture publicly.

Contained Reset, Slipping Rollout

The wage reset remains contained but SDPR runs into compounding slippage across successive LHD waves: training fatigue, super-user burnout, infrastructure variability, public confidence erosion. The Hunter New England pattern repeats and intensifies, the program timeline to end-2028 becomes politically expensive to hold, and NSW Health workforce attention is consumed by sequential digital-deployment stabilisation rather than by the structural retention agenda. The FY27 re-base lands but the structural pipeline reforms in Cluster 4 are deferred to FY28. The strategic posture pivots to SDPR workforce-stabilisation as the binding constraint on every other workforce decision.

Early indicators: a second LHD slip after HNE; super-user attrition surfacing in Q4 2026; SDPRIA publishing a revised state-wide timeline.

Cascade Reset, Slipping Rollout

The wage-precedent cascade and SDPR slippage compound. NSW Health is running a structural cost-base shock at the same time as one of the State's largest digital-deployment programs loses cadence, and the leadership bandwidth available for the AI-mandate and scope-of-practice agendas narrows to maintenance level. The FY27 plan is functionally re-opened mid-cycle; the Ministry's industrial-relations function becomes the central coordinating node for several years; SDPR completion slips beyond end-2028. The strategic posture is triage: protect Tranche B and the ACCHO MBS opportunity, defer the ambient-AI-scribe posture, accept that pipeline reform is an FY28-FY29 agenda.

Early indicators: two named signals from Scenario 2 plus the indicators from Scenario 3 surfacing in the same quarter; Ministry-level reorganisation of program governance; explicit Treasury contingency drawdown.

What We Are Not Planning For

A standalone climate-resilience workforce program in this cycle

Climate-resilience workforce planning remains a real and rising obligation, but it does not sit inside the FY27 decision window the four Strategic Implications address. It is better served by a dedicated cycle later in the year that can carry the depth NSW Health Disaster Preparedness, NSW Resilience and the LHDs warrant, rather than being folded into a workforce-financial-base cycle dominated by the IRC reset and SDPR.

Reinstate if: a NSW-scale climate-driven workforce disruption event surfaces in 2026, or the NSW Treasury 2026-27 Budget allocates explicit climate-resilience workforce capital that requires immediate workforce-planning response.

A dedicated Aboriginal Health Workforce strategy cycle inside this brief

The Aboriginal Community Controlled sector is treated here as a snapshot lens and as a cluster sub-spotlight, with the NACCHO MBS update foregrounded. A full Aboriginal Health Workforce strategic-foresight cycle, with consultation depth across Lowitja Institute, NACCHO, AH&MRC and NSW Aboriginal Health, deserves its own brief in the next quarter and was held out deliberately to avoid presenting it as a subordinate workstream.

Reinstate if: NSW Health publishes a refreshed Aboriginal Health Workforce strategy, or NACCHO and AH&MRC signal a material new federation-level workforce priority during 2026.

The pay-and-conditions industrial-relations detail beneath the IRC award

The detailed industrial-relations mechanics of the staged 16/18/28% award, the enterprise-bargaining sequencing, the back-pay administration, and the LHD-by-LHD payroll implementation belong with the Ministry's industrial-relations function and with HR Shared Services. The strategic-foresight register treats the IRC decision as a workforce-economics shock and a precedent question; the operational mechanics sit outside the brief by design.

Reinstate if: implementation difficulties with the staged award surface as a strategic risk to the FY27 budget cycle, or if a downstream NSWIRC matter escalates the IR detail into a strategic-positioning question.

Digital-health workforce questions beyond SDPR

SDPR is the single largest digital deployment in the cycle, and the right anchor for the digital-workforce conversation this brief carries. The wider digital-health workforce question, covering My Health Record adoption, NSW Health virtual care platforms, secondary uses of clinical data, and the data-and-analytics workforce build, is held back so that SDPR can be treated cleanly and so that the wider digital-health cycle can carry the depth it requires.

Reinstate if: a Commonwealth digital-health workforce program with NSW funding lines is announced, or NSW Health publishes a digital-health workforce strategy refresh during 2026.

Discussion Points for the Leadership Team

1. At what cumulative cost does NSW Health concede that the year-one 10% RN reset will not hold attrition, and what is the explicit trigger that moves SI 4 from Monitor to Decide?
2. Does the inter-cadre relativity compression (AIN 28% vs RN 16%) change how LHDs should design skill-mix from FY27, or is the Ministry's posture to hold current ratios and let the relativities re-equilibrate through enterprise bargaining?

3. If the Hunter New England SDPR cut-over slips publicly into July-August 2026, does NSW Health pause Tranche B, or push through to maintain program cadence at the cost of additional workforce strain?
4. Who owns the de facto NSW Health ambient-AI-scribe standard if Future Health Workforce Planning does not decide one by end-Q3 2026: LHD procurement, individual clinicians, or the indemnity market?
5. Does NSW Health resource ACCHOs to act on the 1 March 2026 MBS reset as a workforce-multiplier, accepting that this is a small budget line with disproportionate Closing the Gap delivery credibility, or treat the change as a Commonwealth MBS matter without NSW workforce implication?

Source Confidence Register

This cycle draws on 43 verified sources across the five clusters, organised under a strict three-month recency window with three named structural anchors (OECD May 2025 digital and AI skills; CRANApplus January 2025 pre-budget submission; HETI 2025-26 Performance Agreement; the Cormack Review October 2024; the NSW 2025-26 Budget; AHPRA WRAP February 2025; Australian Health Review June 2025 longitudinal analysis) and one evergreen regulator hub (AHPRA AI guidance) explicitly identified as such. The Tier 1+2 share is high in Clusters 1 (NSW IRC primary decision, NSW Government and Treasury, NSWNMA), 3 (OECD, TGA, AHPRA, Allens, RACGP/AJGP) and 4 (Productivity Commission, AHPRA, NSW Government, ANMF, CRANApplus, HETI, peer-reviewed journals). Cluster 2 (SDPR) leans more heavily on Tier 3 specialist trade press because frontline-readiness signals come from on-the-record clinician sources reported by Pulse+IT and Health Services Daily; those signals are corroborated by the NSW Health and Justice Health primary disclosures of the cut-over itself. Three Health Services Daily items are paywalled and are treated as directional even where they are not formally flagged as such. The most material source conflict is between SDPRIA's own acknowledgment that "critical foundations" are still being laid and eHealth NSW's training-program scale data; the report weights the candid acknowledgment more heavily than the training-throughput numbers, on the reading that throughput does not guarantee site-level readiness.

Source tiers: **Tier 1**, governments, regulators and intergovernmental bodies. **Tier 2**, think-tanks, academic institutes, major consultancies and quality data providers. **Tier 3**, quality journalism and specialist trade press. **Tier 4**, vendor, company and practitioner sources, used only as directional corroboration.

Cluster 1: The Wage Reset

Source	Tier	Date	Key claim used
NSW IRC, NSWIRComm 4 decision announcement	Tier 1	Apr 2026	Staged 16/18/28% award over three years from 1 July 2025; year-one resets 10/12/22% plus 3% in each of years two and three; finding of historical undervaluation tied to gendered assumptions about caring and interpersonal skills.
NSW Government joint ministerial release	Tier 1	Apr 2026	NSW Government acceptance of the IRC decision with back-pay from 1 July 2025; framed as the largest single uplift in NSW nursing pay in history.
NSW Treasury 2025-26 Budget overview	Tier 1	Jun 2025	\$9.5 million for an internal NSW Health locum agency inside a \$35.1 billion health envelope (structural anchor outside the 3-month window).
NSWNMA, "Mixed emotions over pay deal"	Tier 2	Apr 2026	Union counter-narrative: historic in scale but does not go far enough; year-one 10% reset will not meet cost-of-living or retention pressure; RNs remain behind other states.
ANMJ coverage of the IRC decision	Tier 3	Apr 2026	Justice Taylor's reasoning citing the ~90% female composition of the workforce and historical wage-setting failure to value caring and interpersonal skills.
Hospital + Healthcare, "Mixed emotions"	Tier 3	Apr 2026	Cross-cadre relativity compression with AINs receiving 28% over three years against RNs 16%; skill-mix economics framing for LHDs.
NSW Liberal Party Shadow Health statement	Tier 3	Apr 2026	Political-risk framing: Opposition welcomes rise, warns against funding through cuts to regional services or capital programs (treated directionally as partisan source).
6 St James Hall case note	Tier 3	Apr 2026	Industrial-law characterisation as a work-value case; precedent risk for other female-dominated NSW public-sector classifications.

Cluster 2: SDPR go-live and the digital-deployment workforce shock

Source	Tier	Date	Key claim used
NSW Health, SDPR program delivery	Tier 1	Evergreen	Rollout sequence: Justice Health and NSW Health Pathology John Hunter Lab on 25 March 2026; HNE LHD and HNE Pathology May 2026; state-wide across 17 LHDs and specialty networks by end-2028.
NSW Government, Justice Health SDPR go-live	Tier 1	Mar 2026	Confirmed first SDPR go-live at Justice Health NSW and NSW Health Pathology on 25 March 2026.
NSW Health Pathology go-live confirmation	Tier 1	Mar 2026	John Hunter Hospital Lab go-live, significant workflow transition and intensive multi-month training program preceding cut-over.
Pulse+IT, SDPR begins go-lives	Tier 3	Mar 2026	More than 25,000 staff in scope across HNE LHD, Justice Health and HNE Pathology; one of the largest coordinated digital deployments NSW Health has attempted.
Health Services Daily, HNE not ready leak	Tier 3	Mar 2026	Internal briefing notes dated 11 March 2026 acknowledging the build is not fully complete, training shortfalls and cybersecurity concerns (paywalled, directional).
Health Services Daily, "bin fire"	Tier 3	May 2026	Frontline reports of scant communication, poor training and missing guidance; Dr Teresa Anderson reframing go-live to "toward the middle of the year" (paywalled, directional).
Health Services Daily, "critical foundations"	Tier 3	Mar 2026	SDPRIA on-record acknowledgment that critical foundations are still being laid; workforce-readiness will be live across the 2026-28 sequential cut-over (paywalled, directional).
eHealth NSW training program (via Pulse+IT)	Tier 3	Feb 2026	Around 1,900 classes completed and more than 21,000 staff trained ahead of HNE go-live; around 200 LHD volunteers supporting training delivery.

Cluster 3: AI-augmented clinical work moves from pilot to mandate

Source	Tier	Date	Key claim used
OECD, Digital and AI Skills in Health Occupations	Tier 1	May 2025	Around 55.5 million online job postings (Canada, UK, US, 2018-2023): fastest-growing digital-skill demands are health information management, telehealth and cybersecurity; augmentation dominates automation risk (structural anchor).
OECD, Scaling AI in Health	Tier 1	Mar 2026	AI used across all OECD health systems but national-level scale limited; only around 10% of medical-imaging applications at national scale; binding constraints are data fragmentation, regulatory uncertainty and workforce-capacity gaps.
TGA, mandatory healthcare-facility reporting	Tier 1	Mar 2026	From 21 March 2026 mandatory healthcare-facility reporting of medical-device adverse events, including AI-enabled software-as-a-medical-device; deaths or serious deteriorations reportable within 10 days.
AHPRA AI in healthcare guidance	Tier 1	Evergreen	Practitioners remain professionally accountable for AI-augmented decisions; human judgment, informed consent, privacy and indemnity compliance.
Allens, "Same rules, smarter tools"	Tier 2	Apr 2026	TGA's updated guidance applies medical-device obligations whenever AI software is intended to influence, inform or replace clinical decisions; "scope creep" as AI products iterate through retraining.
RACGP / AJGP, "Is AI A-OK?"	Tier 2	May 2025	Medicolegal considerations for GPs using AI scribes: consult indemnity insurers, review outputs before sign-off, emerging case law shaping standard of care.
RACGP newsGP, AI scribe use poll	Tier 3	Mar 2026	GP AI-scribe use 22% (August 2024) to 40% (November 2025); only 25% of practices have an AI governance policy; only 42% of GPs know their indemnifier's AI position.
Medical Republic, "How many GPs are actually using AI scribes?"	Tier 3	Mar 2026	Independent corroboration of around 40% practice-level adoption; binding constraints are governance, workflow integration and PMS integration.

Source	Tier	Date	Key claim used
Healthcare IT News (ANZ)	Tier 3	Apr 2026	Focus groups with 33 Australian medical practitioners: insufficient AI knowledge, medico-legal risk, privacy concerns primary barriers to ambient scribe adoption.

Cluster 4: Pipeline and retention

Source	Tier	Date	Key claim used
Productivity Commission, RoGS 2026 Health	Tier 1	Feb 2026	Medical-practitioner attrition 24.2% (from 22.9% in 2023); nurses and midwives 24.5% (down from 28.9%); 122,749 FTE medical practitioners; 372,821 FTE nurses and midwives.
AHPRA Workforce Retention and Attrition Project	Tier 1	Feb 2025	Across nine regulated professions, 79% intend to maintain registration, more than 12% unsure or intend to leave; top drivers include mental burnout, feeling undervalued, work no longer fulfilling (structural anchor).
NSW Government, Rural Generalist 2026 intake	Tier 1	Feb 2026	24 additional doctors into the NSW Rural Generalist Single Employer Pathway 2026 intake across eight rural LHDs.
ANMF Pre-Budget Submission 2026-27	Tier 2	Jan 2026	Calls for federally mandated minimum staffing in NSQHS Standards; financial-and-time burden on student nurses during clinical placements as binding pipeline constraint.
CRANaplus Pre-Budget Submission 2025-26	Tier 2	Jan 2025	Pipeline plan for 1,000 nationally accredited generalist rural-and-remote nurses and midwives by 2031 (structural anchor).
HETI 2025-26 Performance Agreement	Tier 2	Jul 2025	\$8.4 million scholarships to 1,842 students; \$0.7 million rural and regional scholarships to 232 students (structural anchor).
Australian Health Review, peer-reviewed retention analysis	Tier 2	Jun 2025	Across nine professions 2014-2023: 29.6% increase in registered practitioners per 100,000; replacement-rate fluctuations (structural anchor).

Source	Tier	Date	Key claim used
Journal of Nursing Management, Dillon et al.	Tier 2	Feb 2026	Region of employment is a material driver of intention-to-remain; mentorship, reflective practice, resilience training and career coaching show strongest retention effects, particularly rural and remote.
ANMJ, "ANMF priorities 2026"	Tier 3	Feb 2026	ANMF 2026 priorities: mandated minimum staffing, scope-of-practice expansion, Maternity Futures Report recommendations.

Cluster 5: Scope-of-practice unfreezes; multidisciplinary care economics

Source	Tier	Date	Key claim used
Department of Health, Cormack Review final report	Tier 1	Oct 2024	18 recommendations across workforce design, development, education and planning; virtually all primary-care health professionals face restrictions to working at full scope unrelated to education or competence (structural anchor).
NACCHO, 1 March 2026 MBS updates	Tier 1	Mar 2026	1 March 2026 MBS updates remove age-based clinical activities from Aboriginal and Torres Strait Islander health-assessment items 715, 218, 92004, 92011; patient-end support for specialist video consultations updated.
MBS Online, Aboriginal and Torres Strait Islander health-assessment factsheet	Tier 1	Mar 2026	Items 715, 218, 92004, 92011 effective 1 March 2026: reframe assessment around clinical need across the life course.
ACN Pre-Budget Submission 2026-27	Tier 2	Jan 2026	Cormack-implementation priorities: NP and endorsed-midwife direct referral; legislate RN prescribing; authorise diagnostic ordering; remove collaborative-arrangement requirement.
AMA, GP-led team-based care position	Tier 2	Apr 2026	Expanded practice-nurse role within GP-led clinical governance: Workforce Incentive Program expansion, practice-level MBS items, career pathways; reframes scope debate around GP-led teams.

Source	Tier	Date	Key claim used
RACGP, measured approach to NT pharmacist scope	Tier 2	Mar 2026	Measured, evidence-based approach to expanded pharmacist scope; GP collaboration and clinical governance over rapid retail-pharmacy autonomous prescribing.
NSW ACI, Rehabilitation Model of Care	Tier 2	Mar 2026	Evidence-based framework for equitable multidisciplinary rehabilitation; multidisciplinary teams central to the future state-wide model.
Croakey, scope-of-practice and Aboriginal communities	Tier 3	Mar 2026	ACCHOs as best-practice multidisciplinary model; around 30% of nurses report not working to full scope; discrimination and cultural-safety barriers remain.
Croakey, treasurers' interest in scope reform	Tier 3	Feb 2026	State and federal treasurers' direct fiscal interest in implementing scope-of-practice recommendations; Cormack reframed as a national-productivity reform.